

FAX COVER

To: Dr. Alan Nguyen (Primary Care) | From: Hematology Clinic

Re: **Jessica Wilson** — AML (IDH1+) ongoing therapy

Date: 15.03.2024 | Pages: 3 | Confidential

CLINIC FOLLOW-UP NOTE

Identifiers: MRN AML008. DOB 25.08.1961. AML diagnosed **03.01.2023**, ELN 2022 **Intermediate**. **IDH1** mutation detected (R132H); FLT3 negative; normal cytogenetics.

1L Regimen (ongoing): Ivosidenib 500 mg PO daily + Venetoclax ramp (100→200→400 mg) then 400 mg PO daily **D1-21 q28 days**. Start date **10.01.2023**. Antimicrobial prophylaxis per ANC.

Interval History (last 3 months): Doing well, gardening again. No fevers, mucosal bleeding, or dyspnea. Reports early satiety days 1-7 of venetoclax cycles; constipation controlled with senna. QTc stable on serial ECGs (range 442-456 ms). No syncopal episodes.

Physical Exam (15.03.2024):

- Vitals: BP 118/72, HR 76, afebrile, BMI 26.2.
- HEENT: no scleral icterus; oral mucosa intact.
- CV/Resp: RRR, clear lungs.
- Abdomen: soft, non-tender; no HSM.
- Skin: no petechiae; old PICC site well healed. ECOG 0.

Laboratory Data:

- CBC: WBC 3.9 (ANC 1.4), Hgb 11.6 g/dL, Plt 167.
- CMP: Na 138, K 4.2, Cr 0.86, AST 22, ALT 18, Tbili 0.6, uric acid 3.1.
- LDH 178.
- Coag: INR 1.0.
- ECG: QTcF 450 ms (unchanged).
- IDH1 R132H digital PCR: **undetectable** in peripheral blood (02.2024).

Bone Marrow (Aug 2023 and Jan 2024): Hypocellular to normocellular marrow with **CR** morphology; blasts <2%; MRD by flow negative. No new clonal evolution on NGS panel.

Comorbidities & Concomitant Meds:

- Paroxysmal atrial fibrillation (rate-controlled). **Apixaban** was held during early thrombocytopenia and then **resumed 2.5 mg bid** once platelets >100; no bleeding episodes since. **Metoprolol 25 mg bid**.
- GERD (omeprazole 20 mg qAM).
- Allergic rhinitis (cetirizine PRN).
- Vaccinations up to date.

Toxicities/Supportive Care:

- Myelosuppression, mostly grade 2 neutropenia near end of cycles: **pegfilgrastim D22** used intermittently.
- TLS: none after initial ramp; maintained hydration.
- Drug-drug interactions considered (azole prophylaxis avoided to maintain venetoclax dosing; used **atovaquone** for PJP prophylaxis during deepest neutropenia).

Assessment:

- **AML, IDH1-mutated** — in continued remission on combined ivosidenib + venetoclax since **10.01.2023** with good tolerance, no clinical/hematologic relapse.

Plan:

1. Continue regimen unchanged (ivosidenib daily; venetoclax D1–21). Consider shortening venetoclax to D1–14 if recurrent grade ≥ 3 neutropenia.
2. CBC weekly for first half of cycle, then mid-cycle; CMP monthly. ECG q8–12 weeks given QTc neutrality so far.
3. Infection prophylaxis tailored to ANC; consider levofloxacin if ANC <0.5 for >7 days.
4. Anticoagulation: maintain apixaban 2.5 mg bid; hold if Plt <50.
5. Next marrow in ~June 2024 or sooner if cytopenias persist.
6. If loss of response emerges, options discussed previously: **venetoclax-hypomethylating agent switch**, clinical trial, or IDH1 inhibitor continuation with alternative backbone; transplant evaluation only if disease biology/drift warrants.

Patient Education: neutropenia precautions; when to call for fever >38°C; avoid grapefruit; check any new meds for CYP3A4 interaction.

Follow-up: Return in 4 weeks; labs locally acceptable.

Signed,
[Name], PA-C / supervising MD